



AROXICAM

(Meloxicam Tablets
7.5 mg and 15mg)

Composition:

Meloxicam Tablets 7.5mg

Each uncoated tablet contains
Meloxicam Ph. Eur. 7.5 mg

Meloxicam Tablets 15mg

Each uncoated tablet contains:
Meloxicam Ph. Eur. 15 mg

Description:

Meloxicam Tablets 7.5mg

Light yellow, round, uncoated tablet with score line between 'F' and '1' debossed on one side and plain on the other side

Meloxicam Tablets 15 mg

Light yellow, round, uncoated tablet with score line between 'F' and '2' debossed on one side and plain on the other side

Pharmacodynamics:

Meloxicam is a non-steroidal anti-inflammatory drug (NSAID) of the oxicam family, with anti-inflammatory, analgesic and antipyretic properties.

The anti-inflammatory activity of Meloxicam has been proven in classical models of inflammation. As with other NSAIDs, its precise mechanism of action remains unknown. However, there is at least one common mode of action shared by all NSAIDs (including Meloxicam): inhibition of the biosynthesis of prostaglandins, known inflammation mediators.

Pharmacokinetics:

Absorption

Meloxicam is well absorbed from the gastrointestinal tract, which is reflected by a high absolute bioavailability of 89% following oral administration.

Following single dose administration of Meloxicam, mean maximum plasma concentrations are achieved within 2 hours for the suspension and within 5-6 hours with solid oral dosage forms.

With multiple dosing, steady state conditions were reached within 3 to 5 days. Once daily dosing leads to drug plasma concentrations with a relatively small peak-trough fluctuation in the range of 0.4 - 1.0 µg/mL for 7.5 mg doses and 0.8 - 2.0 µg/mL for 15 mg doses, respectively (C_{min} and C_{max} at steady state, respectively). Maximum plasma concentrations of Meloxicam at steady state are achieved within five to six hours. Continuous treatment for periods of more than one year results in similar drug concentrations to those seen once steady state is first achieved. Extent of absorption for Meloxicam following oral administration is not altered by concomitant food intake.

Distribution

Meloxicam is very strongly bound to plasma proteins, essentially albumin (99%). Meloxicam penetrates into synovial fluid to give concentrations approximately half of those in plasma.

Volume of distribution is low, on average 11 L. Interindividual variation is the order of 30-40%.

Biotransformation:

Meloxicam undergoes extensive hepatic biotransformation. Four different metabolites of Meloxicam were identified in urine, which are all pharmacodynamically inactive. The major metabolite, 5'-carboxy Meloxicam (60% of dose), is formed by oxidation of an intermediate metabolite 5'-hydroxy methyl Meloxicam, which is also excreted to a lesser extent (9% of dose).

Elimination

Meloxicam is excreted predominantly in the form of metabolites and occurs to equal extents in urine and faeces. Less than 5% of the daily dose is excreted unchanged in faeces, while only traces of the parent compound are excreted in urine.

The mean elimination half-life is about 20 hours. Total plasma clearance amounts on average 8 mL/min.

Indications:

Meloxicam is indicated for relief of the signs and symptoms of
Osteoarthritis
Rheumatoid arthritis
Ankylosing spondylitis

Recommended Dosage:

Meloxicam may be taken without regard to timing of meals.

Osteoarthritis

7.5mg per day. If necessary the dose may increased to 10 mg/day.

Rheumatoid arthritis

15 mg/day. According to therapeutic response, the dose may be reduced to 7.5 mg/day

Ankylosing spondylitis

In patients with increased risks of adverse reactions: Start treatment at the dose of 7.5 mg/day.

In dialysis patients with severe renal failure: The dose should not exceed 7.5 mg/day. The total daily dosage of meloxicam should not exceed 15mg. As a dosage for used in children has yet to be established, usage should be restricted to adults. Tablets should be swallowed with water or other fluid in conjunction with food.

Elderly: In elderly patients being treated for rheumatoid arthritis the recommended dose for long term treatment is 7.5 mg a day.

Children (Under 15years of age): Not recommended

After assessing the risk/benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used.

Route of Administration:

For oral administration only.

Contraindications:

Meloxicam is contraindicated in the following situations:

Pregnancy and lactation

Hypersensitivity to Meloxicam or to one of the excipients or hypersensitivity to substances with a similar action, e.g. NSAIDs, aspirin. Meloxicam should not be given to patients who have developed signs of asthma, nasal polyps, angioneurotic edema or urticaria following the administration of aspirin or other NSAIDs.

Active gastro-intestinal ulcer or history of recurrent gastro-intestinal ulcer;

Severely impaired liver function;

Non-dialysed severe renal failure;

Overt gastro-intestinal bleeding, recent cerebrovascular bleeding or established systemic bleeding disorders.

Severe uncontrolled heart failure

Children under 12 years, except when approved for use in Juvenile Rheumatoid Arthritis.

Warning and Precautions:

As with other NSAID's caution should be exercised when treating patients with a history of gastro-intestinal disease and in patients receiving treatment with anticoagulants. Patients with gastro-intestinal symptoms should be monitored. Meloxicam should be withdrawn if peptic ulceration or gastro-intestinal bleeding occurs.

Gastro-intestinal bleeding, ulceration or perforation can occur at any time during treatment, with or without warning symptoms or a previous history of serious gastro-intestinal events. The consequences of such events are generally more serious in the elderly.

NSAIDs inhibit the synthesis of renal prostaglandins which play a supportive role in the maintenance of renal perfusion. In patients whose renal blood flow and blood volume are decreased, administration of an NSAID may precipitate overt renal decomposition which is typically followed by recovery to pretreatment state upon discontinuation of non-steroidal anti-inflammatory therapy. Patients at greatest risk of such a reaction are dehydrated patients, those with congestive heart failure, liver cirrhosis, nephritic syndrome and overt renal disease; those receive a diuretic or those having undergone major surgical procedures, which led to hypovolemia. In such patients the volume of diuresis and renal function should be carefully monitored at the beginning of therapy. The dose of meloxicam in patients with end-stage renal failure on haemodialysis should not be higher than 7.5 mg. No dose reduction is required in patients with clinically stable liver cirrhosis. Frail or debilitated patients may tolerate side effect less well and such patients should be carefully supervised. As with other NASIDs, caution should be used in the treatment of elderly patients who are more likely to be suffering from impaired renal, hepatic or cardiac function.

Induction of sodium potassium and water retention and interference with the natriuretic effects of diuretics may occur with NSAIDs. Cardiac failure or hypertension may be precipitated or exacerbated in susceptible patients as a result. There are no specific studies about effects on the ability to drive vehicles and to use machinery. Patients who experience visual disturbances, drowsiness or other central nervous system disturbances should refrain from these activities.

Undesirable effects may be minimised by using the lowest effective dose for the shortest duration necessary to control symptoms. The recommended maximum daily dose should not be exceeded in case of insufficient therapeutic effect, nor should an additional NSAID be added to the therapy because this may increase the toxicity while therapeutic advantage has not been proven. The use of Aroxicam with concomitant NSAIDs including cyclooxygenase-2 selective inhibitors should be avoided.

In the absence of improvement after several days, the clinical benefit of the treatment should be reassessed. Any history of esophagitis, gastritis and/or peptic ulcer must be sought in order to ensure their total cure before starting treatment with meloxicam.

Attention should routinely be paid to the possible onset of a recurrence in patients treated with meloxicam and with a past history of this type.

Functional renal failure:

NSAIDs, by inhibiting the vasodilating effect of renal prostaglandins, may induce a functional renal failure by reduction of glomerular filtration. This adverse event is dose-dependant. At the beginning of the treatment, or after dose increase, careful monitoring of diuresis and renal function is recommended in patients with the following risk factors:

- Elderly
- Concomitant treatments such as ACE inhibitors, angiotensin-II antagonists, sartans, diuretics
- Hypovolemia (whatever the cause)
- Congestive heart failure
- Renal failure
- Nephrotic syndrome
- Lupus nephropathy
- Severe hepatic dysfunction (serum albumin <25 g/l or Child-Pugh score 10')

In rare instance NSAIDs may be the cause of interstitial nephritis, glomerulonephritis, renal medullary necrosis or nephrotic syndrome.

Sodium and water retention:

Sodium and water retention with possibility of oedema, hypertension or hypertension aggravation, cardiac failure aggravation. Clinical monitoring is necessary, as soon as starting therapy in case of hypertension or cardiac failure. A decrease of the antihypertensive effect can occur.

Induction of sodium, potassium and water retention and interference with the natriuretic effects of diuretics and consequently possible exacerbations of the condition of patients with cardiac failure or hypertension may occur with NSAIDs.

Hyperkalaemia: Hyperkalaemia can be favoured by diabetes or concomitant treatment known to increase kalaemia. Regular monitoring of potassium values should be performed in such cases.

Adverse reactions are often less well tolerated in elderly, fragile or weakened individuals, who therefore require careful monitoring. As with other NSAIDs, particular caution is required in the elderly, in whom renal, hepatic and cardiac functions are frequently impaired. The elderly have an increased frequency of adverse reactions to NSAIDs especially gastrointestinal bleeding and perforation which may be fatal. Meloxicam, as any other NSAID may mask symptoms of an underlying infectious disease.

The use of meloxicam, as with any drug known to inhibit cyclooxygenase / prostaglandin synthesis, may impair fertility and is not recommended in women attempting to conceive. In women who have difficulties conceiving, or who are undergoing investigation of infertility, withdrawal of meloxicam should be considered.

Patients with rare hereditary problems of galactose intolerance, the Lapp-lactase deficiency or glucose-galactose malabsorption should not take this medicine.

A/s: 145 x 300 mm  Black

 AUROBINDO Packaging Development		Product Name	Component	Item Code	Date & Time
		Aroxicam	Leaflet	P1541720	12.02.2026 & 12.10 pm
		Customer / Country	Version No.	Reason Of Issue	Reviewed / Approved by
Malaysia	00	Revision			
Team Leader	Kiran Kumar	Dimensions	No. of Colours : 01  41720		
Initiator	Arun	145 x 300 mm			
Artist: MJ Designers Rajitha	Pharmacode	41720			
Additional Information : Supersede item Code: P1530968					Sign / Date

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